

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.058**

SUBJECT: Admissions

POINT PERSON: Admissions Director

APPROVED: Sara Walker, MD

DATE: JUNE 24, 2024


Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. This policy establishes Oregon State Hospital (OSH) policy and procedures for patient admissions.
- B. This policy applies to all staff.

II. POLICY

- A. Oregon state hospital admits patients 18 years of age and over per ORS (426.130, 426.701, 427.290, 179.473, 161.315, 161.327, 161.328, 161.365, 161.370, 419C.530, 419C.378, 420.505).
- B. Oregon State Hospital (OSH) will admit individuals who meet criteria for commitment to OSH in a safe and timely manner and as bed become available. All admissions must follow processes in this policy and in pertinent Oregon Administrative Rules (OAR) and Oregon Revised Statutes (ORS).
- C. Upon receipt of an admission referral to OSH, the Admissions Director must ensure that there is a legal authority for the admission. If there is not, the Admissions Director must notify the requesting parties in writing with reference to the appropriate authority.
- D. An Admissions RN must perform a brief assessment for every patient admitted to OSH including, at minimum, vital signs and visual observation to identify any medical needs requiring immediate attention before the patient leaves the Admissions area. If a patient is too combative to safely obtain vital signs, this must be communicated with Unit and documented in the medical record along with the nature of the patient's behavior.

- E. If a newly admitted patient experiences an emergency medical condition, OSH must provide emergency care as indicated in OSH Policy 8.002 in accordance with regulations and licensure until the patient can be transferred to an acute care facility.
- F. Individuals who are identified prior to admission as having significant medical concerns must be referred to the Chief of Medicine or their designee for review per Procedures A.
- G. The Admissions Director must prioritize admissions as follows:
 - 1. If the person is a civil admission who does not meet the expedited criteria, they will be placed on the waitlist
 - 2. PSRB revocations and expedited civil admissions will be scheduled for direct admission, according to availability of bed space.
 - 3. Court ordered commitments will be prioritized based on their legal status and admitted subject to availability of bed space.”
- H. For patient-type specific admissions requirements and placement guidelines, staff must refer to Procedures B.
- I. The Oregon Health Authority (OHA), or designee, will determine whether OSH will admit an individual who is civilly committed, who requests voluntary commitment, whose guardian requests commitment for the individual (voluntary by guardian [VG]), or whose health care representative (HCR) requests commitment for the individual (voluntary by HCR).
- J. Psychiatric Security Review Board (PSRB) community clients may be voluntarily admitted if the individual needs OSH services, but conditional release status is not revoked. If such an individual has the capacity to consent to voluntary admission to OSH (or has a personal representative who may consent on their behalf), the individual may apply for voluntary admission to OSH.
 - 1. OSH’s determination of whether to accept the application will be subject to review for appropriateness of admission and is dependent upon bed availability. Admissions must review the admission request on the following:
 - a. Need for inpatient services at the Oregon State Hospital and no other alternative is likely to be successful, with endorsement of this need, in writing, by the PSRB.
 - b. Availability of a bed for the likely duration of their stay.
 - c. Approval by the Chief Medical Officer or the Superintendent.
 - 2. If OSH accepts a voluntary admission application from a PSRB

client, the client may remain at OSH for no more than 30 days, unless the client, the PSRB, and OSH agree that circumstances require a longer stay.

- J. OYA (Oregon Youth Authority) 30-Day Administrative Transfer request under Interagency Agreement; OYA Transfers to OSH.
 - 1. An Interagency transfer of a youth from OYA can be made upon request from an OYA Authorized Representative as outlined in the Interagency Agreement between OYA and OSH.
 - 2. Transfers from OYA require CMO, or designee review, and approval as outlined in the Interagency Agreement between OYA and OSH.
- K. The Admissions Director must coordinate transportation of the patient to the State Hospital if a sending facility cannot transport a patient on the scheduled day, and the delay will result in the individual not being admitted. The Admissions Director must track this internally.
- L. Admissions shall occur during regular business hours on non-holidays.
 - 1. Holiday and after-hours admissions are limited to PSRB patients whose conditional release status has been revoked and who the PSRB has deemed require immediate admission.
- M. The Admissions Director must:
 - 1. Schedule admissions for the coming week based on the Admissions List
 - 2. Assign incoming patients to Admissions staff to conduct initial screening which includes; medical concerns, the collection of documentation and to obtain provider to provider verbal handover.
- N. The Admissions Director or designee shall assign accepted individuals to receiving units based on the best fit between their clinical needs and bed availability. Safety of patients and safety of staff is paramount, and no patient will be admitted to a unit that does not have the necessary resources to provide safe care.
- O. In cases where the Admissions Team is not certain which type of program will best meet an individual's needs, they may consult the Supervising Psychiatrist or Program Director for the program identified as most likely to be appropriate. If unavailable,

Admissions may contact the Chief of Psychiatry or their designee.

- P. Assigned staff must complete applicable commitment- specific admission tasks as indicated in Admissions task checklists and in other department protocols and OSH policies and procedures.
- Q. In accordance with OAR 333-510-0060, at minimum each patient must be provided the following items including, but not limited to:
 - 1. a good bed, mattress, pillow, and bed linens.
 - 2. personal care items; and
 - 3. separate storage space for items, including personal belongings that are within OSH guidelines of policy 8.037.
- R. Admissions staff, to the extent possible, must seek the following information from the sending facility and document the information in the electronic medical record:
 - 1. medical history, review of health systems;
 - 2. medication reconciliation with diagnosis, dosages, allergies, and side effects, including copies of the medication administration record;
 - 3. vital signs, including respiration, pulse, temperature, height, weight, blood pressure in the last 24 hours;
 - 4. physician examination;
 - 5. test results that have been done,
 - 6. electrocardiogram (EKG), if clinically indicated;
 - 7. reports of any blood work, urine analysis, or x-rays which were clinically indicated and ordered;
 - 8. purified protein derivative (PPD) results if the patient has stayed in jail more than two weeks or was an inpatient more than two weeks;
 - 9. copy of consultation reports, if any;
 - 10. copy of emergency department evaluation form; and
 - 11. discharge summary (when available) when coming from an inpatient setting.
- S. On a quarterly basis, the Admissions Director must review the appropriateness of placement of recent admissions with the Chief of Psychiatry and self-selected representatives from Program Executive

Teams whose units receive admissions.

- T. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services, Shared Services, and Oregon Health Authority policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- U. Staff who fails to comply with this policy or related procedures may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Admission" means actions completed by OSH admissions coordinators and admitting staff before and during a committed individual's arrival at OSH designed to coordinate safe and timely admission of the individual.
- B. "Admission Director" refers to the person responsible for coordinating admitting patients to OSH during normal business hours and after hours.
- C. "Civil commitment" refers to individuals who have been civilly committed to the Oregon Health Authority under ORS 426.130.
- D. Emergency medical condition in accordance with 42 Code of Federal Regulations § 489.24 means:
 - 1. a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain, psychiatric disturbances and/or symptoms of substance abuse) such that the absence of immediate medical attention could reasonably be expected to result in –
 - a. placing the health of the individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy;
 - b. serious impairment to bodily functions; or
 - c. serious dysfunction of any bodily organ or part; or
 - 2. with respect to a pregnant woman who is having contractions –
 - a. that there is inadequate time to affect a safe transfer to another hospital before delivery; or
 - b. that transfer may pose a threat to the health or safety of the woman or the unborn child.
- E. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).
- F. "OYA" Oregon Youth Authority

G. Staff” includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

Procedures A Admission of Patients with Medical Problems

Procedures B Admissions

V. RELATED OSH POLICIES AND PROTOCOLS

Oregon State Hospital Policy and Procedure Manual. *Admission of patients with medical problems*, 6.049. Author.

Oregon State Hospital Policy and Procedure Manual. *Contraband and prohibited items*, 8.044. Author.

Oregon State Hospital Policy and Procedure Manual. *Emergency care provided by OSH*, 8.002. Author.

Oregon State Hospital Policy and Procedure Manual. *Medication reconciliation*, 6.035. Author.

Oregon State Hospital Policy and Procedure Manual. *Patient property and valuables*, 8.037. Author.

Oregon State Hospital Policy and Procedure Manual. *Release of information and communication with patient families, guardians, significant others*, 6.021. Author.

Oregon State Hospital Policy and Procedure Manual. *Suicide risk screening and assessment*, 6.056. Author.

Oregon State Hospital Policy and Procedure Manual. *Trauma screening*, 6.052. Author.

VI. REFERENCES

Joint Commission Resources, Inc. (2018). The joint commission comprehensive accreditation manual for hospitals, PC.01.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rule § 309-035-0215.

Oregon Administrative Rule §§ 309-091-0010 – 309-091-0025.

Oregon Administrative Rule § 333-505-0030.

Oregon Administrative Rule § 333-510-0045.

Oregon Administrative Rule § 333-510-0010, 0060.

Oregon Administrative Rules § 333-525-0000.

Oregon Revised Statutes § 161.315.

Oregon Revised Statutes §§ 161.327 — 161.328.
Oregon Revised Statute § 161.336.
Oregon Revised Statutes §§ 161.365 — 161.370.
Oregon Revised Statute § 420.505.
Oregon Revised Statute § 426.130.
Oregon Revised Statute § 426.220.
Oregon Revised Statute § 426.225.
Oregon Revised Statutes §§ 426.232 — 426.234.
Oregon Revised Statutes §§ 426.650 — 426.675.
Oregon Revised Statute § 426.701.
Oregon Revised Statute § 441.155.